

ST. JUDE NEUROLOGICAL INSTITUTE & ADVANCED BRAIN CLINIC

Department of Clinical Neuroscience • Comprehensive Video-EEG Telemetry & Surgical Resection Assessment

Patient Name:	Eleanor Vance	Patient Code / MRN:	PT-8942
Date of Birth:	1970-04-12 (Age 54)	Gender:	Female
Primary Diagnosis:	Refractory Temporal Lobe Epilepsy	Attending Physician:	Dr. Sarah Jenkins, MD
Evaluation Date:	2026-08-18	Admission Unit:	Epilepsy Monitoring Unit (EMU)

CHIEF COMPLAINT & HISTORY OF PRESENT ILLNESS

Patient Eleanor Vance is a 54-year-old female with a 3-year history of medically refractory focal impaired awareness seizures with temporal lobe semiology. Seizure manifestations consist of stereotypic olfactory auras (burnt rubber sensation) accompanied by epigastric rising, followed by behavioral arrest, left-handed fidgeting, and oro-alimentary automatisms lasting 60–90 seconds with post-ictal speech disturbance.

ANTISEIZURE POLYTHERAPY & MEDICAL HISTORY

Current Antiseizure Medications: Levetiracetam (Keppra) 1500 mg PO BID, Lamotrigine 200 mg PO BID, Lisinopril 10 mg daily for hypertension.

Allergies: Penicillin (severe anaphylaxis and angioedema).

Past Medical History: Essential Hypertension (controlled, BP 124/78 mmHg).

LONG-TERM CONTINUOUS VIDEO-EEG FINDINGS (72-HOUR TELEMETRY)

- Baseline Background: Continuous symmetrical 9.5 Hz posterior dominant alpha rhythm.
- Interictal Abnormalities: Frequent epileptiform sharp and slow wave discharges localized exclusively to the left anterior-mesial temporal electrodes (F7-T3, T3-T5).
- Ictal Recordings: Two stereotypic electroclinical seizures captured. Ictal onset demonstrated rhythmic 5 Hz theta activity originating from the left mesial temporal region, confirming definitive electroclinical concordance.

NEUROPSYCHOLOGICAL & LABORATORY PANEL

- Laboratory Panel: Serum sodium 139 mEq/L, potassium 4.1 mEq/L, BUN 14 mg/dL, creatinine 0.85 mg/dL, AST 22 U/L, ALT 24 U/L, WBC 6.8 x10³/uL, Platelets 242 x10³/uL.
- Neuropsychology: Mild verbal memory encoding deficit with intact visual-spatial perception and preserved executive reasoning.

MULTIDISCIPLINARY ASSESSMENT & SURGICAL RECOMMENDATIONS

Multidisciplinary Epilepsy Surgical Conference consensus recommends patient as an excellent candidate for left anterior temporal lobectomy or stereotactic Laser Interstitial Thermal Therapy (LITT) given complete electroclinical concordance. Scheduled for surgical planning consultation in 4 weeks.